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Name: إبتسام محمد حواس عبدالعز: National ID: 28808010113728	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
محافظة تحقيق التنمية

الاسم

محمد حواس عبدالعزير

١٧ د ١٧ في الثاني - احمد عصمت

عن شمس - القاهرة

٢٨ ١٣٧ ١٠ ٠٨ ٠٨ ٢٨

HX4969002



٢٠١٩/٠٢

٢٨٨٠٨٠١٠١٣٧٢٨

مدير ادارة رعاية الطلاب

مدرسة المجد الحديثة الخاصة

مفتحة

مسلية

أنقى

محمود عسر احمد سيد احمد

البطاقة سارية حتى ٢٠٢٤/٠٢/٢٤





مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المصرية
صحة
مصر

MDT FORM

9 / 13 / 2022 G 700
C. Co

Name	المنشع ع	MRN	٨٣٩٦٣٥٩٤
		ID	١٠٠٦٦٩٩٩٤١
Age	36	Phone	٠١٠٦٦٦٠٢٨٣
Menopausal status	Premen	Marital status	٢٨٨٠٨٠١١٢٧٧
NRS SCORE		Family history	
BMI		REASON OF MDT	BRG
CO morbidity			

Clinical Data

BX: Invasive Mucopapillary Car

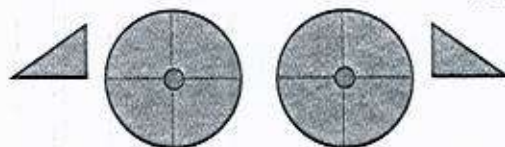
R T3N

L

ER: 8/8 PR: 8/8 HER 2: re. Ki67:

T2N3

Sonammammography: Bilateral



TNM

Mets Workup:

Recommendations: Bilateral. Dr 2024 → NACH AC Xu → Tamoxifen. Shift to Tamoxifen 16 to Xu → RT MRM T3N2. then 11/2024 → Rx PET/CT Combined Breast. + Biopsy → 1st Metastasis 12/2024 T2N3 → PET/CT MRM + DM.

MDT OUTCOME & PLAN (NB) during NACH success TAM then shift to AJ during Combined Side till Nov. For - Adj. AZI Zoladex & consider Extended Adj. Hormonal th. Bilateral PORT. prophylactic tamoxifen 16 months.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Amr Elsay		Amr Elsay			

(Handwritten: لاور)

Patient Name
DOB
DOB
IC No

12007755
01-08-1968
2000000000000000

Location
Sex

External Org
Female

Specimen Id
Accession No
Sample Type

2412000000
P-24-15512

Surgical Pathology-Diagnosis
Biopsy

Surgical Pathology-Diagnosis
Biopsy

Collection Date 07/12/2024 10:21 AM
Received Date 07/12/2024 10:21 AM
Doctor

CLINICAL HISTORY
S/P right NACTH and MRM (P-24-13015), for invasive microglandular carcinoma, grade 2,
diagnosed upon biopsy (12821/23), presenting with left breast mass UIQ, core biopsy.

GROSS DESCRIPTION
Fragmented cores

MICROSCOPIC DESCRIPTION

Fragmented cores

HISTOLOGIC DESCRIPTION

- Histologic type: Invasive duct carcinoma
- Histologic grade: Grade II (Intermediate)
- Ductal carcinoma in situ (DCIS): Not identified
- Histologic grade: Grade II (Intermediate)
- Carcinoma: Not identified
- Lymphovascular invasion: Not identified
- Perineural invasion: Not identified
- Special studies (including hormone receptor):
 - ER positive, showing moderate positive reaction
 - PR positive, showing moderate positive reaction
 - HER2 is negative, score 0

- MACROSCOPIC DESCRIPTION**
- Histologic type - Invasive duct carcinoma
 - Histologic grade - Grade II (Intermediate)
 - Ductal carcinoma in situ (DCIS) - Not identified
 - Lymphovascular emboli - Not identified
 - Preductal invasion - Not identified
 - Special studies (including hormone receptor):
 - ER is positive, showing moderate positive reaction for 65% of tumor cells, score 2/8
 - PR is positive, showing moderate positive reaction for 35% of tumor cells, score 6/8
 - Her2/neu is negative, score 0
- DIAGNOSIS**
- Left breast

Left breast mass ULQ, core biopsy : Invasive duct carcinoma, grade 2.

Malignant primary site (single tumor)
Assisting pathologist

Dr. Shiman Abdelaziz
Signed by:

Dr. Shumaa Zakaria, MD (Lecturer)

Reported and electronically endorsed by
Human Addictive

Specimen Name	12302/25	Location	External Clinic
DOB	01/08/1981	Sex	Female
ID No	2480000000000000		

Surgical Pathology-Diagnosis

Specimen Id 2480000000000000
Accession No P25-172
Sample Type TISSUE

Collection Date 05/01/2025 08:54 AM
Received Date 05/01/2025 10:12 AM
Doctor

CLINICAL HISTORY

Modified radical mastectomy for invasive micropapillary carcinoma (path: 17621/23).

GROSS DESCRIPTION

1. MAM specimen 23x20 cm covered by skin ellipse 16x15 cm. Areola and nipple are both present and remarkable. Serial sectioning revealed a fairly defined greyish white firm mass 3.5x2x1 cm. Least deep margin of excision is 2 cm. Largest dissected axillary lymph node is 3x2 cm.

HISTOPATHOLOGIC DESCRIPTION

Sections examined revealed an invasive carcinoma formed of irregular small solid groups of malignant ductal epithelial cells lie within artifactual stromal spaces cased by shrinkage of surrounding stroma. With focal ductal differentiation. Anaplasia and mitosis are mild to moderate. Lymphovascular invasion is noted. Nipple is free of tumour cells and Paget's disease. Least deep margin of excision is negative for tumour spread. Ten of the nineteen dissected lymph nodes are positive for metastatic spread (10/19) with evidence of extra-nodal extension.

COMMENT

DIAGNOSIS

Lt breast, modified radical mastectomy :
Invasive micropapillary carcinoma, grade 2.

PREPARY

Malignant primary site (single tumor)

Signed by
Dr. Mostafa Ahmad, MD (L)

Pathologist
Dr. Abdelwahab Elghareeb

Registrar

Reported and electronically endorsed by: Hani Abdelwahab

Print Date: 05/01/2025 01:40 PM

Print Date: 05/01/2025 12:25 PM

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Name: Mrs. Ebtesam Mohamed Hawas
Date: 25/12/2023
Ref. by: Prof. Dr. Mai Kamal Fakhry

**POST-CONTRAST MDCT OF THE CHEST, ABDOMEN AND PELVIS
REVEALED:**

I- CHEST:

- The right breast shows mild diffuse skin thickening and is the seat of lobulated enhancing soft tissue mass at its upper outer quadrant, measuring 4 x 4 x 6.5 cm in maximal axial and CC dimensions.
- Several enlarged right axillary lymph nodes with the majority being rounded in shape and lost their fatty hilum. The largest measures about 20 x 20 mm in maximal short and long axes.
- No pulmonary alveolar or interstitial masses or intertraces seen with normal aeration of both lungs.
- No evidence of pleural or pericardial sac collections.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- Normal appearance of the thoracic aorta and the great vessels.



II- ABDOMEN AND PELVIS

- The liver is mildly enlarged and showing smooth regular outline. It shows no homogeneous hypodense parenchyma with no evidence of focal hepatic lesions or biliary dilatation.
- Normal CT appearance of the spleen. No focal lesions. A splenule is seen at the splenic hilum.
- Normal appearance of suparenal glands, pancreas and both well-functioning kidneys.
- Normal CT appearance of pelvic organs. An IUD is seen in the uterus.
- No ascites.
- No pelvic/abdominal lymphadenopathy.
- No evidence of lytic or sclerotic bony lesions.



Name: Mrs. Ebtesam Mohamed Hawas
Date: 25/12/2023
Ref. by: Prof. Dr. Mai Kamal Fakhry

Opinion:

- Right breast mild diffuse skin thickening and lobulated enhancing soft tissue mass at its upper outer quadrant, likely of malignant neoplastic origin, for clinical and sono-mammographic correlation as well as histopathological assessment.
- Several right axillary lymphadenopathies with suspicious criteria, worrisome of being metastatic.
- Mild fatty hepatomegaly with no evidence of hepatic focal lesions.

Best regards,
Dr. Rana Raouf

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vessels.

II- ABDOMEN AND PELVIS

- The liver is mildly enlarged and showing smooth regular outline. It shows homogeneous hypoattenuated parenchyma with no evidence of focal hepatic lesions or ICBRs dilatation.
- Normal CT appearance of the spleen. No focal lesions. A splenule is seen at the splenic hilum.
- Normal appearance of suprarenal glands, pancreas and both well functioning kidneys.
- Normal CT appearance of pelvic organs. An IUC is seen in the uterus.
- No ascites.
- No peri-abdominal lymphadenopathy.
- No evidence of lytic or sclerotic bony lesions.



Name: Mrs. Ebessam Mohamed Hawas
Date: 25 / 12 / 2023
Ref. by: Prof. Dr. Mai Kamal Fakhry

Opinion:

- Right breast mild diffuse skin thickening and lobulated enhancing soft tissue mass at its upper outer quadrant, likely of malignant neoplastic origin, for clinical and sono-mammographic correlation as well as histopathological assessment.
- Several right axillary lymphadenopathies with suspicious criteria, worrisome of being metastatic.
- Mild fatty hepatomegaly with no evidence of hepatic focal lesions.

Best regards,
Dr. Rana Fawzi
Dr. Mona Fawzi



S C A N

Name: Ebtesam Mohamed Hawas

ID: 4188575

Referred by: Prof.

Date: 25/11/2024

High Field Dynamic MRI of Both Breasts

Technique :

- Axial T2 (plate 1).
- Axial STIR (plate 2).
- Sagittal postcontrast 3D TFE of the left breast (plate 3).
- Dynamic multiphase post-contrast study done in 8 minutes with MIP reconstruction (plate 4). Time intensity curves (plate 5).

Findings:

- Unremarkable right mastectomy and right axillary clearance .
- Left breast glandular condensation and evidences of homogenous glandular enhancement.
- Left UIQ minimally speculated outline mass , it measures 44x30x27 mm in diameters , the lesion is low signal in T2 and STAIR with intense mass like enhancement essentially peripheral with central necrosis. The kinetic data obtained indicates a malignant pattern , rapid rise followed by wash out and plateau , type 2& 3 curves .
- Normal skin thickness and contour of the left breast .
- There is multiple likely non-specific left axillary lymphadenopathy.

OPINION :

Unremarkable right mastectomy .

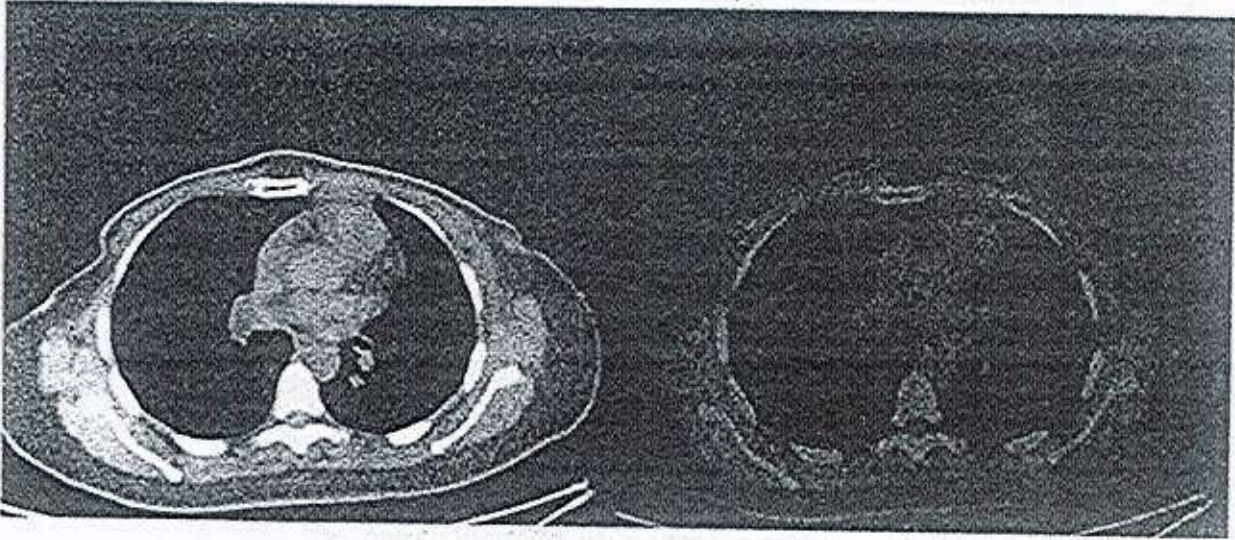
Left breast UIQ malignant mass , categorized as BIRADS 5 based on the morphological and the kinetic data. Microbiopsy is recommended.

Prof. Naglaa Abdel Razek.MD



Findings:-

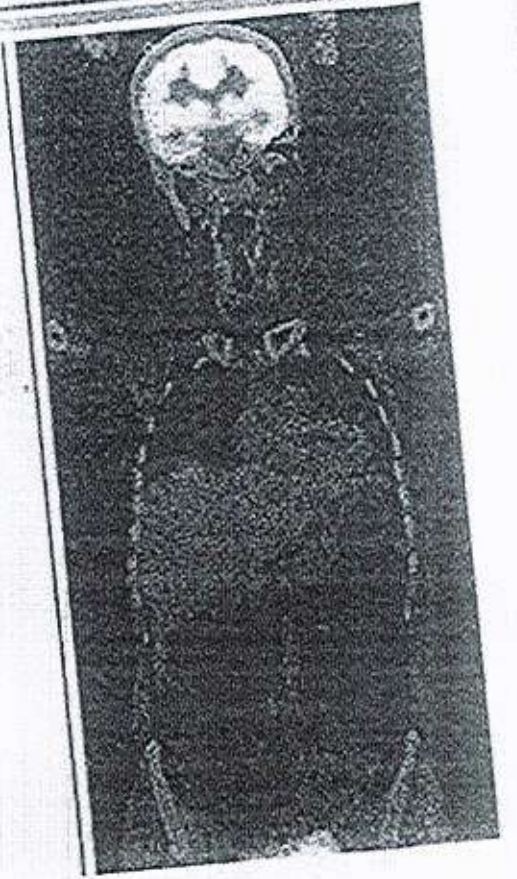
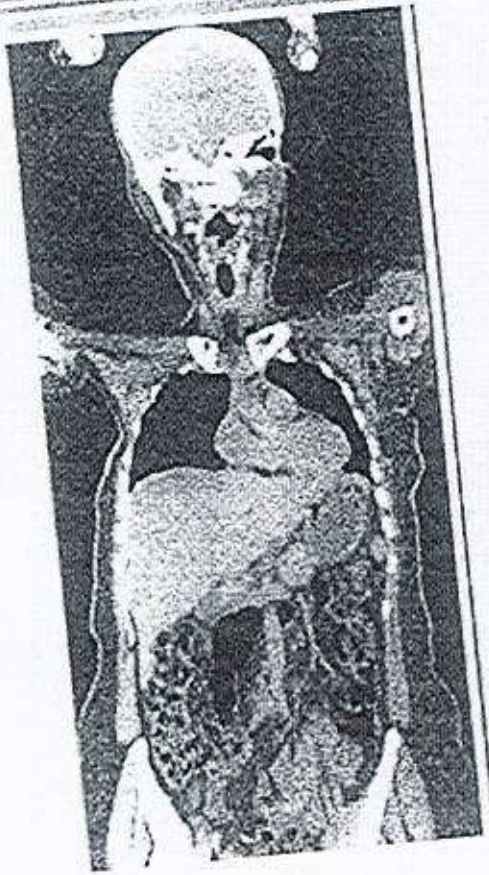
As compared to previous study dated at 20/11/2024 , current study reveals:-



I. Both breasts/1st site of concern):

- Clear right mastectomy bed with no metabolically active (FDG avid) soft tissue lesions that could account for loco regional tumoural residue or recurrence.
- Left MRM bed showing ill defined fibrotic soft tissue sheath with diffuse strandy fat planes & overlying thickened skin of low grade activity ... likely post-operative sequel . No evidence of operative bed metabolically active (FDG avid) soft tissue masses that could account for loco-regional residual or recurrence.
- No metabolically active (FDG avid) bilateral axillary lymph nodes are noted.





II. Head :

- Uniform tracer bio-distribution over the scanned sections of the cerebral and cerebellar hemispheres.
- No metabolically active (FDG avid) masses are noted.

III. Neck:-

- No metabolically active (FDG avid) cervical lymph-nodes are noted.
- Normal physiological tracer distribution over the scanned pharyngeal lymphoid tissue as well as salivary glands.

IV. Chest:-

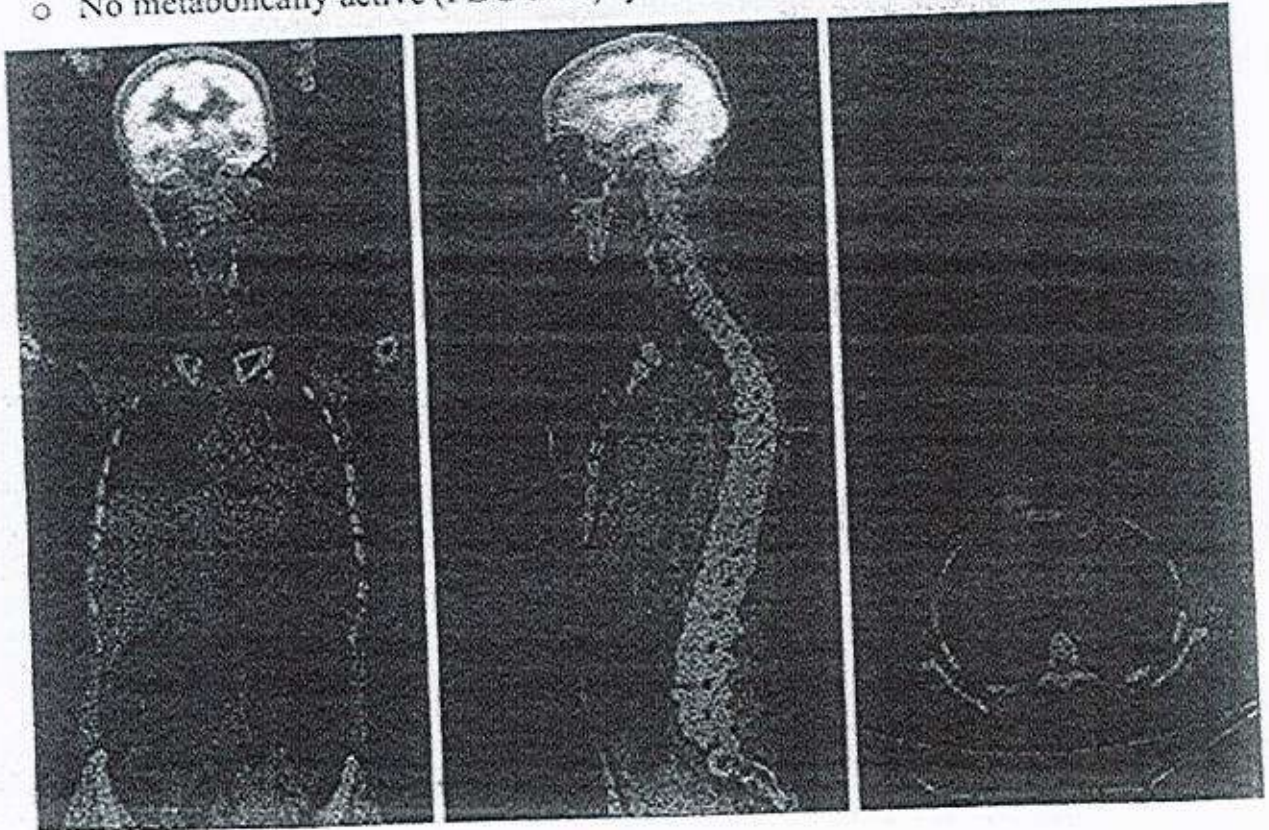
- No metabolically active (FDG avid) pulmonary nodules or masses
- No metabolically active (FDG avid) mediastinal or hilar lymph nodes
- No metabolically active (FDG avid) pleural or pericardial effusion.
- Normal tracer biodistribution of the heart.

**V. Abdomen and pelvis:-**

- Enlarged fatty liver shows no metabolically active (FDG avid) focal lesions or DIHBRs.
- Average size spleen showing no metabolically active (FDG avid) focal lesions.
- No metabolically active (FDG avid) abdominal lymph nodes are seen.
- Normal tracer concentration and excretion at both kidneys, ureters and urinary bladder.
- Normal appearance of the pancreas, aorta and IVC.
- No metabolically active (FDG avid) ascites.

VI. Scanned axial and Appendicular skeleton:-

- No metabolically active (FDG avid) lytic or sclerotic osseous lesions.



**∴ Conclusion: -**

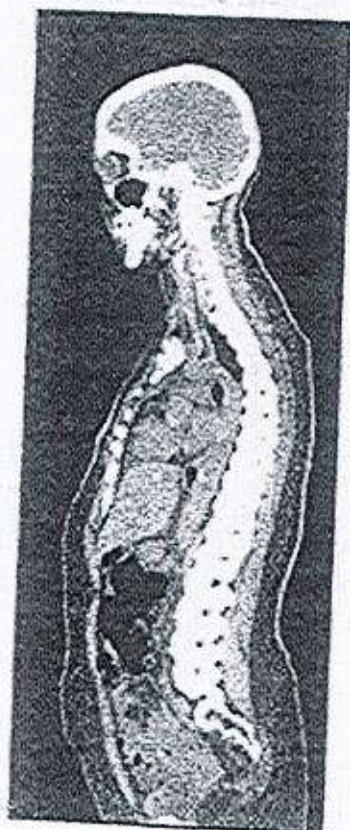
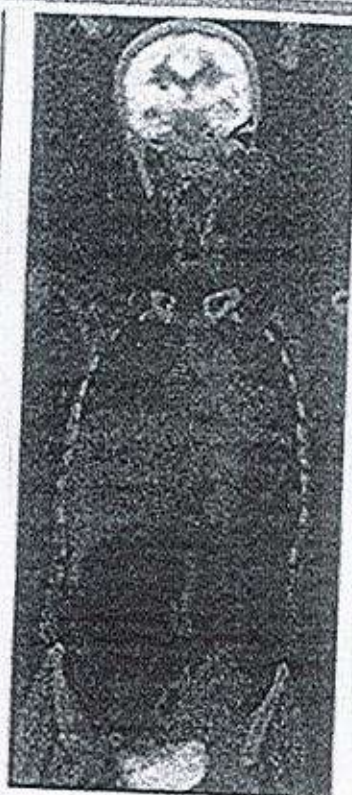
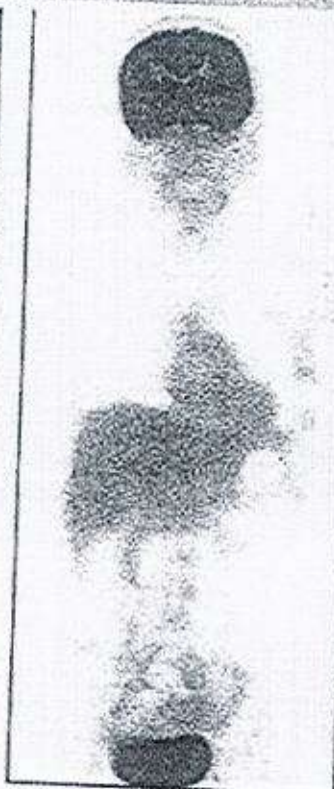
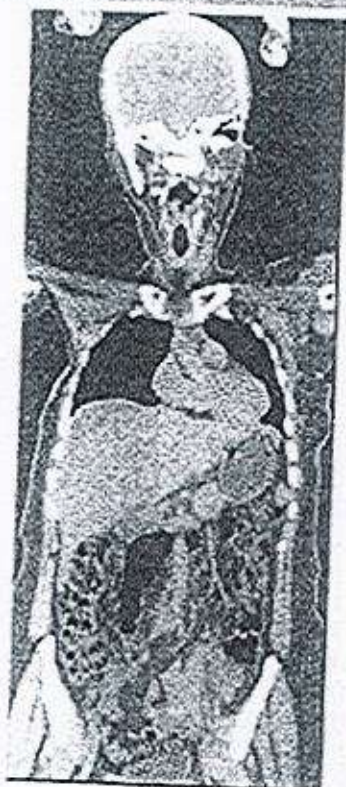
∴ Negative follow up study of a known patient with bilateral breast carcinoma, Status post left mastectomy showing:

- ✓ No metabolically active (FDG avid) right mastectomy bed loco regional tumoural residue/recurrence.
- ✓ Left MRM bed showing post-operative sequel ,however no evidence of metabolically active (FDG avid) loco-regional tumoral residue or recurrence. for follow up.
- ✓ No metabolically active (FDG avid) metastatic deposit.

PET-CT Unit

Dr. Khaled Yousef.
Radiologist.

Dr. Hany Maurice .
Nuclear Medicine Consultant
University of London.



Fused Cervicals